# Chapter 17: Tailoring the TOGAF Standard

*Study Guide, based on Mastering the TOGAF Standard by Eric Jager (2025), Chapter 17 (pp. 713-723)*

## Overview

Chapter 17 (pp. 713-723) brings together the book's recurring message that TOGAF must be adapted to the organization. Factors that drive tailoring include industry regulation and compliance, project scope and complexity, the organization's maturity, and preferred modelling languages and tools (p. 713). Section 17.1 lists five ways to customize the Standard, with a nine-step checklist (pp. 714-718). Section 17.2 applies them to a healthcare case to rebut the claim that TOGAF lacks real-world examples (pp. 719-722). Section 17.3 summarizes (pp. 722-723). Leave with this idea: "The essential scaffolding of the TOGAF Framework is the concepts. Everything else... is either an example or a starter set", so substitute what doesn't fit (p. 715). Chapter 17 is part of the GREEN, BLUE, and GOLD reading tours (p. 713).

> **Reading note:** The quotations about customization come from the Standard. The healthcare case (HIPAA, GDPR, HL7 FHIR, ISO 13485, phase prioritization, regulatory decision gates) is Jager's illustration (pp. 719-722). The metamodel and Content Framework tailoring was covered in Ch. 5.2 and Ch. 6.4. This chapter summarizes all tailoring options in one place.

## Detailed Section Breakdown

### Chapter Opener (p. 713)

- TOGAF states early on that it should be adapted. Drivers: industry regulations, best practice and compliance, project scope and complexity, current maturity, modelling languages, tools, and techniques (p. 713).

### 17.1 Designed for Customization (pp. 714-718)

- "The TOGAF Framework is designed to require interpretation or customization" (p. 714).

| Way to tailor | What can be adapted | Page |
|---|---|---|
| Organizational context | Entities to fit strategy and operating model. Metamodel to industry regulation and compliance. Size and structure (small, medium, large; centralized or decentralized). Governance and processes to culture and maturity | pp. 714-715 |
| The ADM | Modify, omit, reorder, or combine phases (e.g., B, C, D together for a high-level definition). Skip unneeded steps (no new principles or team needed). Integrate with agile (e.g., SAFe). Add governance checkpoints. The Implementation Wheel is an example | pp. 715-716 |
| Content Framework and deliverables | Align the metamodel and Content Framework with proprietary data models. Simplify or expand deliverables to match maturity. Align with ITIL, COBIT, Zachman, ArchiMate, agile (e.g., ITIL processes as functions or capabilities, a stakeholder entity from ArchiMate) | p. 716 |
| Governance and compliance | Add organization-specific policies to the governance structure. Tailor techniques such as Risk and Stakeholder Management | p. 716 |
| Tools and techniques | Choose ArchiMate, UML, BPMN, or custom notations. Integrate EA tools (e.g., Bizzdesign, LeanIX) | p. 716 |

- **Nine-step checklist (pp. 717-718):** align with organizational goals; adapt to industry standards; scale to organization size (large may use the full breadth, small only the parts that add value); reflect

maturity; adjust the ADM; customize the Content Framework and deliverables; change governance and compliance structures; select tools and techniques.

- **Answering critics.** Practitioners abandon TOGAF because it describes *what*, not *how*, and lacks real examples. Section 17.2 provides one (p. 718; see Ch. 1.1).

### 17.2 Tailoring the TOGAF Standard for the Healthcare Industry (pp. 719-722)

- **Healthcare challenges.** Regulatory compliance, interoperability, patient data security, fast-moving technology (p. 719).
- **Licence to adapt.** Architects "may choose not to include entities and relationships... and/or add additional entities and relationships" (p. 719).

| Tailoring area | Healthcare adaptation | Page |
|---|---|---|
| Healthcare-specific needs | Align with HIPAA, GDPR, HL7 FHIR, ISO 13485. Add patient-centric entities to the metamodel. Modify artifacts for patient experience, EHRs, digital health. Align clinical and operational work. Strengthen security architecture | pp. 719-720 |
| ADM | Agile for fast tech adoption (telemedicine, AI). *Phase prioritization*: emphasize B (patient journeys, workflows), strengthen D (cloud, data lakes, AI diagnostics), enhance F (legacy and EHR modernization) | p. 720 |
| Content Framework and deliverables | FHIR-based data models. Capability models (telemedicine, remote monitoring, AI diagnostics; Figure 17-1). Interoperability maps (EHR, lab, pharmacy, PACS/RIS). Risk and compliance models (breaches, AI misdiagnosis, downtime) | pp. 720-721 |
| Governance and compliance | An Architecture Board with clinical, IT security, compliance, and medical informatics members. Regulatory decision gates (e.g., HIPAA risk assessments, AI diagnostic approvals) | p. 721 |
| Tools and techniques | ArchiMate with healthcare extensions. FHIR interoperability testing tools. AI analytics. Predictive modelling of patient flow | pp. 721-722 |

- **Result.** Better patient outcomes, compliance, and interoperability, with focus on security, interoperability, AI, and cloud ecosystems (p. 722).

### 17.3 Summary (pp. 722-723)

- Tailoring is essential to get value. Five customization areas: organizational context, ADM, Content Framework and deliverables, governance and compliance, tools and techniques. TOGAF can be tailored to "virtually any situation, scenario, organization, or industry" (pp. 722-723).

## Key Terms & Definitions

| Term | Definition | Page |
|---|---|---|
| Tailoring | Adapting the TOGAF Standard to the organization's needs, context, and maturity | pp. 713-714 |
| Essential scaffolding | TOGAF's concepts. Everything else is an example or starter set that can be substituted | p. 715 |
| Phase prioritization | Emphasizing certain ADM phases (e.g., B, D, F) to fit industry needs | p. 720 |
| Regulatory decision gate | A governance checkpoint for regulatory approval (e.g., a HIPAA risk assessment) | p. 721 |
| HL7 FHIR | Fast Healthcare Interoperability Resources: rules for secure exchange of electronic health data | pp. 719-720 |

## Figures & Tables Referenced

| Figure / Table | What it shows | Page |
|---|---|---|
| Figure 17-1 | Tailored industry-specific Business Capability Diagram (healthcare) | p. 721 |

## Self-Check Questions

**Q:** Name the five ways the TOGAF Standard can be customized.

**A:** Tailoring for organizational context; tailoring the ADM; tailoring the Content Framework and deliverables; tailoring governance and compliance; tailoring tools and techniques (p. 714).

**Q:** What does the Standard call its "essential scaffolding", and what does that imply?

**A:** Its concepts. Everything else is an example or starter set that can be replaced using the modular structure (p. 715).

**Q:** Give two examples of tailoring the ADM.

**A:** Skip Preliminary steps when no new principles or team are needed; combine B, C, and D for a high-level architecture; reorder phases; integrate SAFe; add governance checkpoints (p. 715).

**Q:** How can ITIL or COBIT be combined with TOGAF content?

**A:** By adding ITIL processes or COBIT parts to the Content Framework as business functions or capabilities (p. 716).

**Q:** In the healthcare case, which ADM phases are prioritized, and why?

**A:** B (patient journeys and hospital workflows), D (cloud, data lakes, AI diagnostics), and F (legacy and EHR modernization) (p. 720).

**Q:** How might an Architecture Board be tailored for healthcare?

**A:** Include clinical representatives, IT security, compliance officers, and medical informatics and health IT experts, and add regulatory decision gates (p. 721).

**Q:** Which factors drive how much TOGAF should be tailored?

**A:** Industry regulations, best practices, and compliance; project scope and complexity; the organization's maturity level; preferred modelling languages, tools, and techniques (p. 713).

**Q:** Why should a small organization not adopt the whole framework?

**A:** Only the parts that add value should be used. Large organizations may implement the full breadth, small ones only parts (p. 717).